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Parenting Research

The Stigmatization of Caregivers of Children With Attention-Deficit/Hyperactivity Disorder: A Systematic Review

Samuel Guerin[1] | Jeffrey McCrossin[2] | Romuald Blanc[3] | Émilie Cappe[1,4]

[1]Laboratory of Psychopathology and Health Processes, Université Paris Cité, Boulogne-Billancourt, France | [2]École de psychoéducation, Université de Montréal, Montreal, Canada | [3]INSERM, Imaging Brain & Neuropsychiatry iBraiN, Université de Tours, Tours, France | [4]Institut Universitaire de France (IUF), Paris, France

Correspondence: Émilie Cappe (emilie.cappe@u-pariscite.fr)

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ABSTRACT

Research on Attention-Deficit/Hyperactivity Disorder (ADHD)-related stigma has centered on individuals with the diagnosis. Evidence concerning caregivers exists but has not been brought together in a way that clarifies its scope, correlates, and implications, despite caregivers' central role in navigating services, supporting daily functioning, and treatment engagement. The present study therefore to systematically review the empirical literature on the stigmatization of caregivers of children with ADHD. Following PRISMA guidelines, we searched PubMed and PsycINFO for studies between 2013 and 2024. Of the 4136 articles, 15 studies examining affiliate and courtesy stigma met criteria. Across studies, affiliate stigma was consistently linked to higher levels of caregiver depression, anxiety, and distress, as well as coping strategies and parenting styles. Higher education levels among caregivers were associated with lower affiliate stigma. The severity of the child's ADHD symptoms and internalizing/externalizing problems significantly associated with the level of stigma experienced by caregivers. Courtesy stigma was associated with several factors, including negative attitudes (e.g., anger or blame directed at children with ADHD) among members of the general public. These findings highlight the multifaceted associations of stigma on caregivers. The association between affiliate stigma and caregiver psychological distress underscores the need for targeted interventions, however only one intervention study met inclusion criteria for this review. The association between negative attitudes and courtesy stigma indicates the importance of addressing public perceptions. Future research should focus on developing and testing interventions that aim to reduce stigma and improve the well-being of caregivers and their families.

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1 | Introduction

Attention-Deficit/Hyperactivity Disorder (ADHD) is a neurodevelopmental disorder characterized by patterns of inattention and/or hyperactivity-impulsivity that interfere significantly with functioning and development (American Psychiatric Association 2022). These symptoms typically manifest in multiple settings (e.g., at home and school) and can lead to significant impairments in academic or social functioning (Cortese et al. 2023). Worldwide prevalence rates of ADHD are estimated at about 5.9% of children, making it the second most common psychological disorder among young people (Faraone et al. 2021).

Despite extensive scientific evidence supporting a genetic and biological etiology for ADHD (Cortese 2012), doubts regarding

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the validity of ADHD as a mental health diagnosis contribute to widespread stigma for individuals with ADHD and their families (Lebowitz et al. 2016).

Stigma may interfere with family dynamics and the effectiveness of family-based interventions. Recent research suggests that ADHD is linked to increased parental stress, harsher or inconsistent parenting practices, and a decline in the quality of the parent–child relationship (Babinski and Sibley 2022). Family therapy has shown promise in improving parenting behaviors, reducing criticism and expressed emotion, and fostering more cooperative behavior in children (Daley et al. 2018; Hornstra et al. 2023). Yet, stigma may hinder caregivers' engagement in these therapeutic approaches. Investigating how stigma affects caregivers' emotional resources, parenting, and participation in behavioral treatments may thus be crucial for optimizing family therapy practices.

Several types of stigma affect families of individuals with ADHD. Public stigma targets individuals through bias and discrimination in schools, workplaces, and clinics, resulting in hurtful communication, exclusion, and denial of opportunities (Mueller et al. 2012). Courtesy stigma affects those associated with individuals with ADHD, like family members and caregivers, often blaming them for the condition (Birenbaum 1970). Internalized stigma, or self-stigma, involves individuals with ADHD adopting negative beliefs about themselves, which are associated with low self-esteem and isolation. Similarly, affiliate stigma is observed among families and caregivers who internalize negative societal perceptions (Papadopoulos et al. 2022). Together, these distinctions provide a conceptual framework for understanding stigma across individuals and their social environments, while highlighting forms of stigma that may also be experienced by caregivers, which are examined more specifically in the present review.

Public stigma has been particularly well-documented within the ADHD literature. This type of stigma relates to societal stereotypes, attitudes, and discrimination toward stigmatized individuals (Corrigan and Kleinlein 2005). Such stigma can be enacted by peers, teachers, or healthcare professionals (Greenway et al. 2023; Metzger and Hamilton 2021; Tatlow-Golden et al. 2016). Public stigma varies across these groups. For instance, healthcare professionals' stigma often involves questioning the validity of ADHD, despite its recognition in international classifications and its initial clinical descriptions dating back to the 18th century (American Psychiatric Association 2022; Van Kernebeek and Crunelle 2024; World Health Organization 2022). Teacher stigma typically results in the exclusion or underestimation of academic abilities of students with ADHD (Metzger and Hamilton 2021; Toye et al. 2019). Peers often exhibit stigma by falsely attributing socially undesirable traits to youth with ADHD, such as being "more violent" and requiring greater social distance (Walker et al. 2008). These factors are associated with poorer social relationships (Pelham and Fabiano 2008), as higher levels of anxiety, depression, and low self-esteem (Becker et al. 2017) in individuals with ADHD.

The difficulties associated with ADHD symptoms often extend to caregivers who may experience courtesy and affiliate stigma due to their close relationship with the child. Caregivers of children with ADHD report lower self-efficacy, self-liking, physical health, psychological health, and poorer social relationships compared to caregivers of children without ADHD (Ahmed et al. 2022). Alongside these notable challenges, there is a significant burden of stigma borne by families, who are often blamed for their child's disorders and receive little social support (Ching'oma et al. 2022; Leitch et al. 2019). Qualitative studies illustrate how stigmatizing experiences manifest for caregivers of children with ADHD. For example, in the study by Leitch et al. (2019), one caregiver described being labeled as lazy and having faced guilt-inducing comments regarding the pharmacological treatment given to their child (e.g., "If you learned to control your kids there wouldn't be a problem"). Other caregivers reported experiencing social isolation and expressed high levels of frustration due to the lack of social support. The internalization of stigma has also been qualitatively documented, with caregivers reporting that they deliberately avoid busy public places or religious services in order to prevent "people looking at us in a certain way," (Schoeman and Voges 2022, 5) reflecting an anticipation of judgment consistent with affiliate stigma. These findings suggest that stigma affects not only the individual child but also extends to the family system as a whole. It is crucial to expand our focus from the individual child and dedicate our attention to fully understanding and addressing how stigma is linked to caregivers.

Both courtesy stigma and affiliate stigma are significantly linked to the mental and emotional well-being of caregivers. Furthermore, affiliate stigma has been found to mediate the negative relationship between general mental health disorder label avoidance and care seeking (Kosyluk et al. 2022). Several studies have indicated that education and knowledge about ADHD are associated with lower levels of public stigma (Ching et al. 2023; Lebowitz 2016; Speerforck et al. 2021). Although public stigma primarily targets individuals with ADHD, it also shapes the broader social climate in which families evolve. These findings therefore underscore the importance of developing a deeper understanding of stigma processes among caregivers in order to better address family needs and to identify the mechanisms that sustain stigma in the general population.

Two previous reviews of ADHD and stigma provide insight into the stigma experienced by caregivers. Lebowitz et al. (2016) found that adults often fail to recognize ADHD as a mental health disorder, resulting in less support and more stigmatizing attitudes toward individuals with ADHD. This stigma extends to the treatments for ADHD, particularly medications, which are often viewed negatively. Parents often face blame for their child's ADHD, which is associated with social isolation and emotional distress. Mueller et al. (2012) found that caregivers experience courtesy stigma, where family members and close associates of individuals with ADHD face social rejection and judgment. Caregivers also internalize this stigma, which is linked to their mental health and quality of life. Widespread misconceptions about ADHD, such as beliefs that it results from poor parenting or lifestyle choices, exacerbate the stigma faced by caregivers. This stigma can negatively affect treatment adherence and the perceived legitimacy of the

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ADHD diagnosis, adding to the caregivers' stress and burden. While these reviews highlight the associations of stigma with caregivers, an up-to-date systematic review dedicated to caregiver stigmatization is needed to provide a clearer overview of how these experiences are described in the literature, particularly in the context of caregivers supporting a child with ADHD, and to identify current challenges and directions for support and intervention.

The aim of this article is to systematically review recent scientific literature on the stigmatization of caregivers of children with ADHD. More specifically, the following objectives will be addressed:

- Objective 1: Report descriptive characteristics of the existing literature on the stigmatization of caregivers of children with ADHD.
- Objective 2: Synthesize knowledge regarding the associations between stigmatization of these caregivers and other relevant outcomes.

2 | Method

This systematic review was conducted according to the PRISMA guidelines (Page et al. 2021). The review protocol was registered with the Prospero database.

2.1 | Eligibility Criteria

Our inclusion criteria encompassed quantitative research, including cross-sectional studies; case–control studies that compared stigmatizing attitudes and experiences between caregivers of children with and without ADHD; prospective cohort studies that followed caregivers over time to examine associations between stigma and various outcomes; and retrospective cohort studies. Similarly, in the case of courtesy stigma, when a hypothetical situation was discussed (e.g., clinical vignette), the concept of ADHD had to be explicitly mentioned and not limited to child characteristics of impulsivity or inattention. Studies were included if ADHD was diagnosed as the primary disorder. Studies including participants with ADHD and comorbidities (e.g., oppositional defiant disorder, anxiety disorders) were not excluded to accurately reflect common complex presentations of the condition (Ottosen et al. 2019). Moreover, to be included in the review, the study had to focus on a homogeneous group of caregivers of children with ADHD. Studies that included participants who were caregivers of children with other neurodevelopmental disorders without intragroup distinctions, for example, were excluded.

2.2 | Information Sources

An electronic search was undertaken with the support of an expert librarian in February 2024 using PubMed and PsycINFO databases, accessed via the National Center for Biotechnology Information, EBSCOhost, and PubPsych platforms. These databases were chosen because they are prominent global sources for psychology and psychiatry research. To refine the search, several criteria were set: the articles had to be (a) in a peer-reviewed journal, (b) published between 2013 and 2024, given the most recent review was conducted in 2013 (Lebowitz 2016), and (c) available in English or French. The search terms focused on ADHD and issues of stigmatization, employing Boolean logic: ((stigma* OR self-stigma* OR stereotyp* OR attitude* OR discrimin*) AND (ADHD OR "attention deficit" OR hyperactiv*)). The query utilized the term truncations like "attention deficit*" and the abbreviation "ADHD," as defined in the DSM-5 (American Psychiatric Association 2022), to capture a wide array of literature related to attention deficit hyperactivity disorder. Consistent with Corrigan and Kleinlein's (2005) definition of public stigma, which includes the dimensions of discrimination, stereotypes, and attitudes, we have incorporated these terms into our algorithms using truncations to gather as many articles as possible.

2.3 | Selection Process

The initial screening of the database findings was performed by the first two authors using the Rayyan online systematic review software (Ouzzani et al. 2016). Duplicate articles were identified and removed prior to title and abstract screening for relevance based on the predetermined inclusion and exclusion criteria. A random sample of 60 articles was selected for independent title and abstract screening by both authors to ensure consistent application of study selection criteria. The two researchers largely agreed on the articles selected for inclusion, with a 98.9% agreement rate resolving any differences through discussion. Articles that passed the initial screening were included for full-text review by the first and second authors. Following the selection process, they also reviewed the references within the chosen articles to uncover any additional pertinent studies related to stigmatization of caregivers of children with ADHD.

2.4 | Data Collection Process

The study characteristics and results were collected by the first author using a data extraction table and checked by the second author, yielding the data displayed in Tables 1 and 2. Omissions or discrepancies were resolved by consensus or, when necessary, involved the third author.

2.5 | Quality and Risk of Bias Assessment Evaluation Criteria

We used the Quality Assessment Tool for Observational Cohort and Cross-Sectional Studies (National Institutes of Health 2014) to evaluate the quality and potential biases of the articles. Consequently, studies were rated as "good" (minimal risk of bias, yielding valid results), "fair" (presence of some bias, yet not significant enough to undermine the results), or "poor" (substantial risk of bias). This evaluation was independently conducted by the first two authors, achieving an inter-rater agreement of 82.4%. Discrepancies were resolved through discussion.

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TABLE 1 | Summary of main characteristics of studies of affiliate stigma.

Author (date)	Country	Study design	Sample size (female %)	Procedure	Quality of evidence[a]
Chang et al. (2021a)	Taiwan	CS	400 (80.3%)	Evaluation after a clinical diagnosis of ADHD, focusing on stress coping strategies and depression in caregivers and behavioral problems in children.	Good
Chang et al. (2021b)	Taiwan	L	382 (80.6%)	Time 1: Evaluation of affiliate stigma and clinical characteristics of parents and the child following a clinical diagnosis of ADHD. Time 2: One-year follow-up evaluation of parental depression levels and children's behavioral problems.	Good
Chang et al. (2020)	Taiwan	CS	400 (76.0%)	Evaluation after a clinical diagnosis of ADHD, focusing on caregivers' attitudes toward children's ADHD and severity of ADHD.	Good
Charbonnier et al. (2019)	France	CS	159 (100%)	Online questionnaire completed by mothers focusing on anxiety and depressive symptoms, self-esteem, life satisfaction and children's ADHD symptoms.	Fair
Chen et al. (2021)	Taiwan	CS	400 (80.2%)	Evaluation after a clinical diagnosis of ADHD focusing on depression and self-esteem in caregivers, caregiver-reported measure of behavioral problems in children and perceived family support.	Good
Chou et al. (2020)	Taiwan	CS	400 (76.0%)	Evaluation after a clinical diagnosis of ADHD, focusing on caregivers' attributed etiologies of ADHD and severity of ADHD.	Good
Chou et al. (2021)	Taiwan	CS	400 (76.0%)	Evaluation after a clinical diagnosis of ADHD, focusing on the employment and rated effectiveness of complementary and alternative intervention strategies by caregivers.	Good
Lin et al. (2023)	Taiwan	CS	213 (78.9%)	Evaluation after a clinical diagnosis of ADHD, focusing on parenting stress and severity of ADHD.	Good
Liu et al. (2023)	Taiwan	CS	213 (78.9%)	Evaluation after a clinical diagnosis of ADHD, focusing on anxiety, depression and hopelessness in caregivers and the severity of ADHD symptoms.	Good
Mikami et al. (2015)	USA	CS	59 (93.6%)	Evaluation after a clinical diagnosis of ADHD, focusing on negative parenting behavior, severity of ADHD, child social skills, child aggressive social behavior, and child social withdrawal.	Fair
Nissley-Tsiopinis et al. (2023)	USA	L	11 (45.5%)	Pre- and post-evaluation of Bootcamp for ADHD (a four-session group intervention designed to prepare parents as informed consumers to engage in multimodal evidence-based treatment for ADHD, whether interventional or pharmacological) for affiliate stigma (other variables were also evaluated). A 6-week follow-up measure was conducted.	Fair
Özaslan and Yıldırım (2021)	Turkey	CS	86 (100%)	Evaluation after a clinical diagnosis of ADHD, focusing on parent self-esteem and ADHD severity.	Fair

Abbreviations: CS, cross-sectional study; L, longitudinal study.
[a]Determined using Quality Assessment Tool for Observational Cohort and Cross-Sectional Studies (National Institutes of Health 2014).

3 | Results

3.1 | Study Selection

Figure 1 outlines the article selection process for this review. Initially, our search identified 4136 articles, from which 1100 were recognized as duplicates. After removing these, 3036 articles underwent a preliminary screening of titles and abstracts based on predefined eligibility criteria. Subsequently, 3014 articles were excluded, permitting the full-text review of 22 articles. Of these, 15 articles were deemed suitable for inclusion. No additional relevant studies were identified after examining bibliographies of the

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articles subjected to full-text screening. The predominant reason for article exclusion was related to the population, specifically the lack of a specific focus on the ADHD population, notably the inclusion of other neurodevelopmental disorders or the absence of a formal ADHD diagnosis. Moreover, two articles were excluded because they did not include a measurement of the stigmatization of ADHD among caregivers of children with ADHD.

3.2 | Study Characteristics

The articles date from 2015 to 2023, of which 14 were published since 2019 (Tables 1 and 2). The research from the 15 articles was conducted in four countries: Taiwan ($n = 8$), United States (USA; $n = 5$), France ($n = 1$), and Turkey ($n = 1$). The studies conducted in Taiwan were produced by the same research team, with at least 75% of their publications reporting on the same sample of individuals. Apart from these studies, the methods differed regarding study design, recruitment, and measure of stigmatization of caregivers of children with ADHD. The study designs were either cross-sectional ($n = 13$) or longitudinal ($n = 2$). In total, three studies measured courtesy stigma while another 12 assessed affiliate stigma. Among the various tools used to assess the stigmatization faced by caregivers of children with ADHD, the Affiliate Stigma Scale (ASS) developed by Mak and Cheung (2008) was the most frequently utilized ($n = 10$ studies). To measure courtesy stigma, researchers consistently used clinical vignettes to present the condition of the child. Recruitment sites were predominantly clinical settings (e.g., hospitals) for studies assessing affiliate stigma (11 out of 12) and primarily academic settings for those measuring courtesy stigma (2 out of 3). Both Sheeper and Vaughn (2021) and Wiley and Vaughn (2020) followed similar research protocols examining stigma onset and the blaming of mothers of children with ADHD through examining public responses to clinical vignettes, yielding highly comparable outcomes.

3.3 | Results of Study Quality and Risk of Bias Evaluation

Seven studies met the previously outlined criteria for "fair" and eight studies (all studies conducted in Taiwan) were classified as "good" (Table 1). The studies conducted in Taiwan evaluated in this systematic review present a low risk of bias, reflecting their rigorous methodological construction and systematic approach to research. They are distinguished by the clarity with which they formulate their research questions, study population, eligibility criteria, and procedures used. A notable strength of these studies lies in the systematic use of validated measurement instruments known for their reliability. Moreover, the high number of participants in these studies, ranging from 213 to 400, allows for the identification of significant effects, particularly for logistic regression. Furthermore, many of these studies included statistical adjustments for potential confounding variables, which enhances the validity of the measured associations, consolidating the foundation on which the conclusions are drawn. Collectively, these elements justify the positive evaluations attributed to these studies, highlighting their capacity to provide reliable and valid data. However, an important caveat is that the same sample was used across most of these studies, which restricts their diversification and may introduce bias.

The other studies included in this review were evaluated as "fair." A recurring factor that led us to identify these studies as presenting a moderate risk of bias is the use of unvalidated measures of stigma (Charbonnier et al. 2019; Özaslan and Yıldırım 2021; Sheeper and Vaughn 2021; Wadian et al. 2019; Wiley and Vaughn 2020). The studies by Sheeper and Vaughn (2021) and Wadian et al. (2019) both relied on a questionnaire developed by Cronan et al. (2016) that had not been validated. Wadian et al. (2019) also relied on an unvalidated measure used in a previous study (Barnett et al. 2015). Charbonnier et al. (2019) and Özaslan and Yıldırım (2021) used tools assessing stigma for psychological disorders in general, which were not validated in their language and were modified to adapt them for ADHD. Regarding recruitment, the study by Charbonnier et al. (2019) included mothers recruited through social networks, without professional involvement to ensure the validity of the ADHD diagnoses reported by the mothers included in the study. Furthermore, the studies by Mikami et al. (2015) and Nissley-Tsiopinis et al. (2023) feature small sample sizes. More broadly, the average percentage of female caregivers in the samples from studies assessing affiliate stigma was 80.9%, which limits the generalizability of the data related to affiliate stigma to all caregivers. Additionally, 13 out of 15 studies were conducted in the USA or Taiwan, which could potentially introduce a cultural bias in the assessment of stigma. Finally, the presence of both cross-sectional and nonexperimental longitudinal designs does not allow for the establishment of causal relationships.

4 | Results

In our review, we found reports of two main types of stigmatization experienced by caregivers of children with ADHD: affiliate stigma and courtesy stigma. A total of 12 studies focused on affiliate stigma (Table 1) while three explored courtesy stigma (Table 2). Studies of affiliate stigma were predominantly cross-sectional studies that examined associations between this phenomenon and other variables including demographics, psychological factors, coping mechanisms, and parenting styles. Articles exploring courtesy stigma used vignettes of children with ADHD to understand public perception of etiology of ADHD and emotions most frequently associated with positive or negative bias toward parents. Main findings related to each type of stigma are summarized below.

4.1 | Affiliate Stigma

4.1.1 | Sociodemographics

Correlations between caregiver educational attainment as well as caregiver and child gender were reported in the studies included in this review. The study by Chang et al. (2020) indicates that a higher level of education among caregivers is associated with lower levels of affiliate stigma. This relationship appears to be robust as it remains positive even when controlling for other sociodemographic variables such as gender, age, marital status, or socioeconomic status (Chang et al. 2020). There were mixed findings regarding the association of gender. Özaslan

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TABLE 2 | Summary of main characteristics of studies of courtesy stigma.

Author (date)	Country	Study design	Sample size (female %)	Procedure	Quality of evidence[a]
Sheeper and Vaughn (2021)	USA	CS	154 (82%)	College students were presented with six vignettes, one of which described a child with ADHD, and then answered questions regarding this child and the child's mother.	Fair
Wadian et al. (2019)	USA	CS	184 (69.0%)	Participants were asked to read descriptions, one at a time, of six hypothetical 10-year-old children, one of whom had ADHD, and then answered questions regarding this child and the child's mother.	Fair
Wiley and Vaughn (2020)	USA	CS	318 (78.0%)	College students were presented with three vignettes, one of which described a child with ADHD, and then answered questions regarding this child and the child's mother.	Fair

Abbreviations: CS, cross-sectional study; L, longitudinal study.
[a]Determined using Quality Assessment Tool for Observational Cohort and Cross-Sectional Studies (National Institutes of Health 2014).

and Yıldırım (2021) found no significant association between parent gender and affiliate stigma, whereas Chang et al. (2020) reported higher levels of affiliate stigma among female caregivers compared to their male counterparts. Furthermore, Chang et al. (2020) also found an association between affiliate stigma and the child's gender, with the stigma being more significant for caregivers of girls, even when controlling for the caregiver sociodemographic factors and the severity and presentation of the child's symptoms of ADHD and Oppositional Defiant Disorder.

4.1.2 | ADHD Presentation and Severity

The severity of the child's ADHD symptoms, rather than the general clinical presentation, was found to vary with caregivers' experience of affiliate stigma. The study by Özaslan and Yıldırım (2021) found no difference in affiliate stigma when accounting for the child's ADHD subtype, comorbidities, and the type of treatment they were receiving. However, several studies indicate that affiliate stigma is positively associated with the severity of ADHD symptoms (Charbonnier et al. 2019; Mikami et al. 2015; Özaslan and Yıldırım 2021). Özaslan and Yıldırım (2021) report that this relationship holds even when controlling for the level of maternal education. Chang et al. (2020) reported similar but more nuanced findings specifically associated with the severity of symptoms related to inattention, demonstrating a positive association with parental affiliate stigma even after controlling for several variables including sex, age, parents' educational level, parents' socioeconomic status, and the sex and age of the children. This was not the case, however, for hyperactivity/impulsivity or oppositional and defiant behaviors. However, Chang et al. (2021b) found that both internalizing problems (as measured by scales of anxiety, depression, withdrawal and somatic complaints) and externalizing problems (including rule-breaking and aggressive behavior) in children were positively associated with affiliate stigma. Finally, the study by Liu et al. (2023) showed that children's inattention severity moderated the effect between affiliate stigma and parental hopelessness and the severity of oppositional defiant disorder moderated the relationship between affiliate stigma and parent–child dysfunctional interaction.

4.1.3 | Caregiver Psychological Factors

Multiple studies demonstrated that the level of caregiver depression correlates with affiliate stigma (Chang et al. 2021b; Charbonnier et al. 2019; Chen et al. 2021). These studies accounted for various factors such as age, sex, family support, self-esteem, the child's internalizing problems, the child's ADHD symptoms, and caregivers' self-esteem. Chang et al. (2021b) found in their study that affiliate stigma was linked to depression in caregivers at 1-year follow-up, but only when other factors like initial depression levels at the time of their child's diagnosis and children's behavioral issues were not taken into account. Additionally, the study by Charbonnier et al. (2019) found that mothers' affiliate stigma scores were positively associated with anxiety scores and negatively associated with life satisfaction and self-esteem. These findings are supported by the study by Liu et al. (2023), which indicates that caregiver affiliate stigma was significantly associated with the severity of caregiver hopelessness. Lin et al. (2023) further demonstrated that affiliate stigma is significantly associated with parental distress and dysfunctional parent–child interactions. Chen et al. (2021) demonstrated that family support, the child's internalizing problems, and the child's ADHD symptoms moderate the relationship between affiliate stigma and depression in caregivers.

4.1.4 | Caregiver Coping and Parenting Styles

Chang et al. (2021a) found that affiliate stigma was positively associated with various coping strategies, such as high problem-focused coping (intensive effort to solve external problems), less seeking of social support (including both instrumental and emotional support), less changing cognition (including positive reinterpretation and acceptance), and a reduced likelihood of

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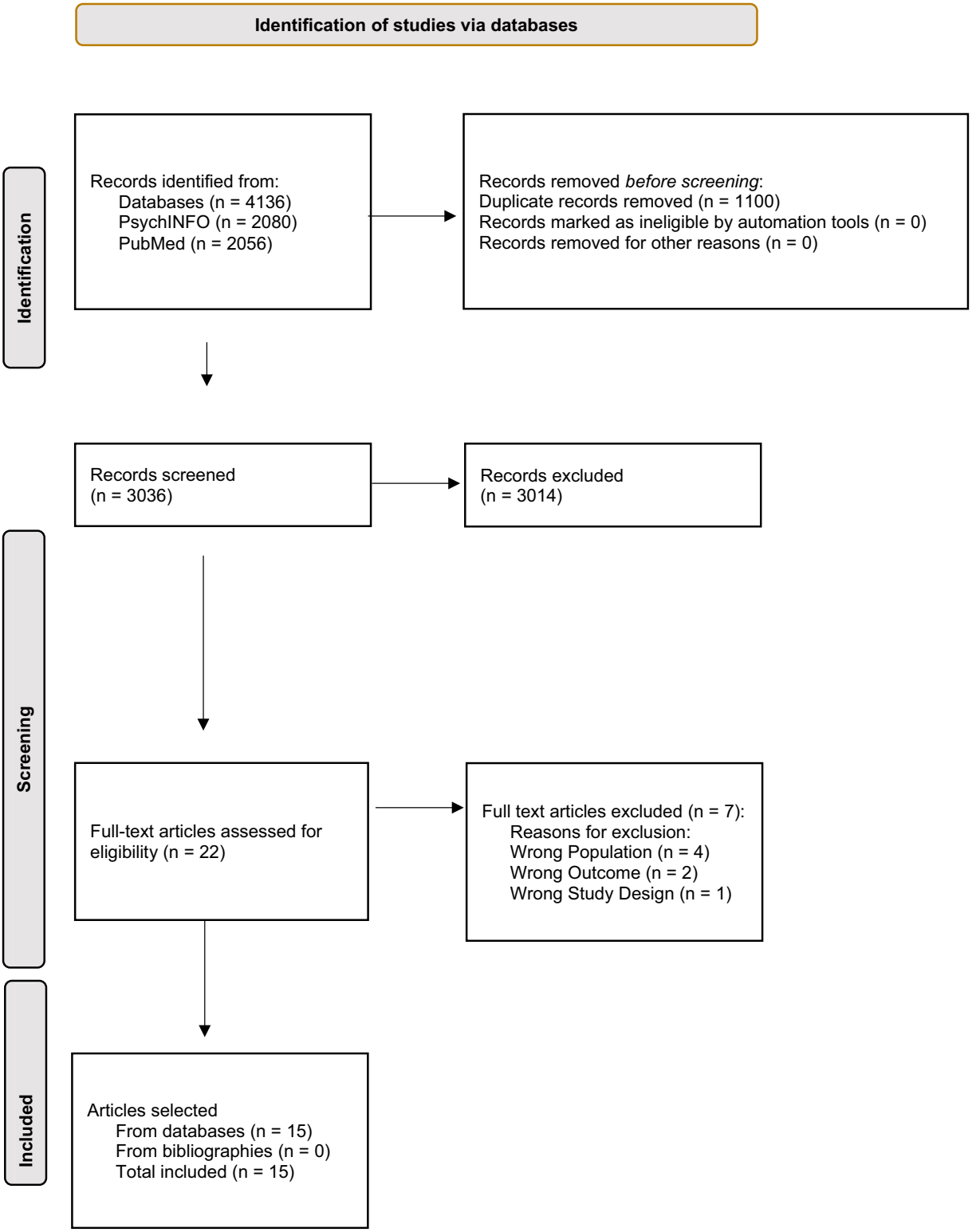


FIGURE 1 | Flow diagram of the selection process following the PRISMA guidelines (Page et al. 2021).

engaging in less effective coping strategies (including denial, and behavioral and mental disengagement). Additionally, the authors found that affiliate stigma is linked to parenting styles such as high care/affection, significant overprotection (overprotective parenting and denial of adolescents' psychological autonomy) and pronounced authoritarian parenting style. Mikami et al. (2015) explored the connection between affiliate stigma and parental negativity (e.g., expressed disparagement, criticism, or contempt toward the child), uncovering a significant correlation between these variables. They found that parental negativity mediates the association between affiliate stigma and child social skills. However, parental negativity was not a mediating factor in the relationship between affiliate stigma and child aggressive social behavior nor child social withdrawal.

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The study by Charbonnier et al. (2019) showed that affiliate stigma partially moderates the relationship between the child's ADHD symptoms and several factors, including the state anxiety of mothers, depression of mothers, life satisfaction of mothers, self-esteem of mothers, and ADHD symptoms of the child.

4.1.5 | Caregiver Understanding and Attitudes Toward ADHD

Moreover, the study by Chang et al. (2020) shows that affiliate stigma is associated with unfavorable attitudes toward diagnosis, pharmacotherapy, behavioral therapy, and biological explanations for etiologies. Additionally, in the study by Chou et al. (2020), caregivers were invited to identify the etiological and causal factors of ADHD. They found that affiliate stigma was positively associated with the causal attribution of several factors such as caregivers' failure to discipline, teachers' failure to discipline, poor diet, poor living environment, an excessive focus on academic success, and attributing causes to supernatural beings or reasons based on divination. The most frequently cited cause was brain dysfunction (84.8%), followed by inadequate caregiver discipline (44.0%); an unhealthy diet, such as high sugar intake (40.8%); a poor living environment (38.8%); children mimicking inappropriate behaviors from peers (37.3%); insufficient discipline by school staff (29.0%); and the education system's strong focus on academic performance (27.3%). These findings demonstrate that extrabiological factors and those inherent to educational factors are often understood by the public as having a causal value in the explanation of ADHD.

4.1.6 | Intervention

The study by Nissley-Tsiopinis et al. (2023) showed that a four-session group intervention designed to prepare parents as informed consumers to engage in multimodal evidence-based treatment for ADHD had no effect on reducing affiliate stigma, both immediately after the intervention and 6 weeks later. However, a large proportion of participants expressed little to no concern about affiliate stigma at baseline, indicating essentially no concern about stigma prior to receiving the intervention.

4.2 | Courtesy Stigma

The emotions expressed by and toward children with ADHD are linked to the courtesy stigma experienced by their caregivers. Research by Sheeper and Vaughn (2021) and Wiley and Vaughn (2020) demonstrated that when individuals from the general public observed a child with ADHD displaying positive emotions in a vignette, they were less likely to blame the mother for the child's condition. Conversely, higher levels of anger directed at the child were associated with increased blame toward the mother. Furthermore, the blame and responsibility the public places on mothers for the child's ADHD are likely linked to their attitudes toward these families. For example, Wiley and Vaughn (2020) further demonstrated that the extent to which respondents believed the mother was responsible for the child's ADHD was associated with the relationship between the perceived likability of the mother and the willingness to offer her personal assistance. In the study by Sheeper and Vaughn (2021), among the six conditions evaluated (ADHD, anxiety, depression, juvenile delinquency, obesity, and fetal alcohol syndrome), stigmatization (responsibility and blame of parents) as well as associations of anger toward the mother were the lowest for ADHD.

In the study by Wadian et al. (2019), the general public rated perceived causes of children's disorders, including ADHD, on a six-point scale. Parental responsibility was rated 2.87 for the onset and 3.29 for the perpetuation of ADHD. These ratings were lower for the children themselves (onset = 2.3; perpetuation = 2.54), whereas biological explanations were rated higher (onset = 4.86; perpetuation = 4.74).

5 | Discussion

This systematic review investigated the stigmatization of caregivers of children with ADHD, incorporating 15 quantitative studies published between 2015 and 2023. To our knowledge, this is the first systematic review to focus exclusively on the stigmatization of caregivers of children with ADHD. The first objective of this review was to report descriptive characteristics of the existing literature. The majority of the articles used samples from either Taiwan or the USA ($n = 13$). The most common type of caregiver stigmatization evaluated was affiliate stigma ($n = 12$). Two studies focused solely on female caregivers, and the samples across all studies were predominantly female (80.9%). Among the included articles, almost all were published since 2019, indicating growing interest in the topic.

The second objective of the review was to synthesize knowledge regarding the associations between stigmatization of these caregivers and relevant outcome variables. Our review highlights several key associations between caregiver stigmatization and caregiver sociodemographic variables, the clinical presentation of ADHD in the child, as well as caregiver coping and parenting styles. Furthermore, the findings related to courtesy stigma suggest that emotions evoked by stories of children with ADHD are linked to both how the public feels about mothers of children with ADHD and the extent to which they blame or hold mothers responsible for their child's condition.

Increased caregiver education was found to be inversely related to affiliate stigma, which in turn was associated with negative attitudes toward ADHD treatments (Chang et al. 2020). However, none of the studies in our review shed light on how parental education and knowledge regarding ADHD are specifically linked to affiliate stigma. Previous studies demonstrated that higher levels of knowledge about ADHD are associated with lower levels of public stigmatization and negative attitudes (Barnett et al. 2012; Kosyluk et al. 2016). While a plethora of parenting interventions for caregivers of children with ADHD exist (Rimestad et al. 2019), we are not aware of any studies that have examined the impact of such programs on affiliate stigma. This may be a particularly interesting approach to addressing affiliate stigma since it is linked to various maladaptive coping strategies and parenting styles, parental negativity and relational difficulties (Chang et al. 2021a; Lin et al. 2023; Mikami et al. 2015) that parenting interventions tend to target (Barkley 2013).

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Affiliate stigma is associated with the severity of the child's ADHD symptoms and with internalizing (e.g., anxiety, depression, somatic complaints) and externalizing problems (e.g., rule-breaking, aggressive behavior; Charbonnier et al. 2019; Mikami et al. 2015; Özaslan and Yıldırım 2021). The relationships between these variables seem to be bidirectional since Mikami et al. (2015) showed that parental negativity mediates the association between affiliate stigma and child social skills, indicating that affiliate stigma could play a role in generating parental negativity and unfavorable outcomes for children.

Our review revealed associations between affiliate stigma and both child and parent gender. Affiliate stigma is more pronounced for parents of girls with ADHD (Chang et al. 2020). This finding could potentially be explained by the higher prevalence of ADHD among boys (Ayano et al. 2020; Pastor et al. 2015) and poorer social acceptability of ADHD diagnoses in girls (Kok et al. 2016). Parents of girls with ADHD may experience greater stigma because the female presentation of the disorder is not normalized in the same way as that of their male counterparts. Furthermore, a meta-analysis demonstrated that parents of girls with ADHD may experience lower levels of stress, suggesting that gender differences in ADHD may be linked to parental experiences through other mechanisms than the symptoms of the disorder themselves (Theule et al. 2013). Although Özaslan and Yıldırım (2021) found no differences in affiliate stigma between genders of parents, mothers and fathers of children with ADHD tend to exhibit differences in parenting styles (El-Deen et al. 2021), which one study in our review indicated is associated with affiliate stigma (Chang et al. 2021a). This suggests that even though direct parent gender-based differences in affiliate stigma might not be evident, differences in parenting approaches may influence how affiliate stigma is experienced. Further to this, positive emotions toward a child with ADHD are associated with lower maternal blame and increased supportive behaviors (Sheeper and Vaughn 2021; Wiley and Vaughn 2020). These findings highlight the systemic interactions of affiliate stigma, showing how public attitudes toward children influence maternal experiences. Shifting public attitudes from blaming mothers to acknowledging their responsibility for their children may encourage supportive behaviors. Together, these findings highlight the need to account for both child and parent sex and gender in future research on affiliate stigma.

To deepen our understanding of these findings, it is useful to consider how some of the included studies drew on theoretical models to frame their methods or interpret their results. Several theoretical models were used to conceptualize the mechanisms underlying caregiver stigmatization. Ecological Systems Theory (Bronfenbrenner 1979) was the most frequently cited model, used to describe the influence of multilevel contextual factors on affiliate stigma. At the mesosystem level, Chang et al. (2021b) found that higher levels of social support are associated with lower affiliate stigma in caregivers, suggesting that interactions between immediate environments (e.g., family and clinical systems) may shape stigma experiences. Chen et al. (2021) also reported that family support and caregiver self-esteem are associated with lower levels of depression in the presence of affiliate stigma, while internalizing symptoms and ADHD severity in the child are associated with higher levels. These associations could be interpreted within the ecological framework as evidence that stigma-related processes unfold at the intersection of individual and relational contexts. Chou et al. (2020) referred to the macrosystem to account for caregivers' preference for complementary and alternative interventions over conventional treatments. This behavioral pattern could reflect an adaptive response to internalized stigma, whereby socially sanctioned alternatives are perceived as more acceptable. Lin et al. (2023) conceptualized affiliate stigma as embedded in exosystemic and macrosystemic forces, reporting that the association between inattention symptoms and caregiver hopelessness is stronger among caregivers reporting higher stigma. This interaction may illustrate the influence of societal norms and expectations on individual psychological outcomes.

The self-stigma model by Corrigan and Watson (2002) served as a theoretical basis for several studies. This model proposes that caregivers may internalize public stigma, which could in turn impact psychological functioning. Charbonnier et al. (2019) drew on this framework to examine the mediating role of affiliate stigma in the association between child ADHD symptoms and maternal depressive symptoms. Mikami et al. (2015) aligned their conceptualization of affiliate stigma with the affective (shame), cognitive (guilt), and behavioral (avoidance) dimensions described by Mak and Cheung (2008). The authors also discussed a limitation of their measure, noting that it does not distinguish between the stages proposed by Corrigan and Rao (2012), such as stereotype awareness, agreement, and self-application. This methodological reflection could encourage the development of more theoretically aligned measurement tools that differentiate perceived stigma from internalized self-devaluation.

Attributional theory (Weiner et al. 1988) was used in studies focusing on courtesy stigma and public attitudes toward parents. Sheeper and Vaughn (2021) found that lower perceived parental responsibility is associated with increased liking and willingness to help, while higher blame is associated with anger and reduced support. These effects were mediated by emotions such as pity and anger, which play a central role in attributional models of social judgment. This pattern could be interpreted as evidence that emotional responses to perceived responsibility shape behavioral intentions toward caregivers. Wadian et al. (2019) reported that when responsibility for ADHD onset was attributed to the child, participants expressed more negative attitudes. Attributions to parents or biological causes were not significantly associated with attitudes. This finding was specific to perceptions of symptom onset, which may suggest that beliefs about the origin of a condition carry more weight in stigma formation than beliefs about its maintenance. Wiley and Vaughn (2020) similarly found that greater parental blame is associated with lower liking of the parents themselves, which in turn is associated with lower willingness to help.

5.1 | Study Limitations

Several limitations should be noted in this review. First, the exclusion of qualitative studies limited findings related to contextual insights and reports of lived experience of caregiver stigmatization. Second, the vast majority of studies included in the review were either from the USA or Taiwan, of which

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the latter predominantly analyzed data from the same sample. Third, there was a significant overrepresentation of female caregivers (81%) in the included studies. Fourth, several studies used unvalidated measures of stigma (Charbonnier et al. 2019; Özaslan and Yıldırım 2021; Sheeper and Vaughn 2021; Wiley and Vaughn 2020). The use of unvalidated instruments is problematic because it undermines the reliability and validity of the data collected, potentially introducing measurement error and limiting the comparability of results across studies and contexts (Boateng et al. 2018).

These factors may limit the generalizability of the findings and could introduce a cultural bias. For example, the availability and use of pharmacological treatments for ADHD vary across countries, shaped by legislation, healthcare infrastructure, and cultural norms (Chappuy et al. 2020; Massuti et al. 2021). These factors not only influence treatment access but also shape the meanings ascribed to ADHD and its medical management, which can affect the degree and form of stigma experienced. As such, generalizing findings of associations between severity of ADHD symptoms and stigma across contexts requires caution (Cortese 2012; Storebø et al. 2015). For example, one study from Taiwan in our review indicated an association between affiliate stigma and unfavorable attitudes toward treatment (Chang et al. 2020). Similarly, a recent study from Saudi Arabia reported that more than half of parents (54%) hesitated or discontinued ADHD pharmacological treatment due to concerns about others' judgments and societal views on medication (Alsubaie et al. 2024). While such findings suggest that stigma can be a barrier to treatment, cross-national differences may limit the extent to which these results can be generalized. Further research is needed that specifically takes sociopolitical contexts into account.

Most studies included in this review relied on self-report questionnaires, which do not distinguish between objective exposure to stigmatizing attitudes and subjective emotional responses. This conflation may limit interpretability, as high stigma scores could reflect either actual negative experiences or caregivers' internalization and emotional reactivity. To address this limitation, future research should also assess stigma as it is expressed in the general population, not only as it is perceived or internalized by caregivers. This dual approach would allow for a more accurate understanding of how societal attitudes contribute to caregiver burden, as illustrated in studies on courtesy stigma (Sheeper and Vaughn 2021; Wadian et al. 2019; Wiley and Vaughn 2020).

5.2 | Clinical Recommendations and Implications for Future Research

This review has highlighted numerous associations between affiliate stigma and variables both in caregivers (levels of depression, anxiety, hopelessness, self-esteem, parenting styles, life satisfaction, coping strategies, parental distress, parent–child dysfunctional interactions, unfavorable attitudes toward diagnosis, etc.) and children (severity of ADHD symptomatology, internalizing and externalizing problems, social withdrawal, social skills, etc.). These findings demonstrate the importance of assessing affiliate stigma in caregivers and proposing psychoeducation methods that could help reduce it. Only one study in this review explored the impact of an intervention on affiliate stigma (Nissley-Tsiopinis et al. 2023), highlighting a significant gap in the literature. We have seen that higher levels of education are associated with lower levels of affiliate stigma (Chang et al. 2020); however, further studies are still needed to determine the effectiveness of targeted parent educational programs related to ADHD on reducing affiliate stigma.

Implicit stigma refers to involuntary, unconscious, and automatic attitudes, behaviors, and emotions directed toward a targeted population (Dovidio et al. 2002). Several studies show that children with ADHD experience implicit stigmatization by their peers, notably through verbal labels such as “dangerous; scary; guilty; weak” (O'Driscoll et al. 2012). However, none of the studies in our review reported on implicit stigmatization among caregivers (Dovidio et al. 2002). This is a significant gap in the literature that should be explored to understand the extent to which this hidden stigma impacts caregivers and how it may be addressed.

The findings from the present review may inform future adaptations of family-based interventions for children with ADHD. While existing programs already aim to improve parenting practices and overall family functioning (Babinski and Sibley 2022; Daley et al. 2018; Hornstra et al. 2023), our review suggests that caregiver stigma—through its associations with less adaptive coping strategies, dysfunctional parenting styles, heightened depressive and anxiety symptoms, and reduced parental self-esteem—may represent an additional target for clinical attention. Stigma-related burden may not only increase parental distress but also reduce treatment engagement, undermine the implementation of parenting strategies, and ultimately limit intervention efficacy. While correlational, these findings suggest that integrating psychoeducational or supportive components focused on stigma reduction and caregiver emotional well-being into family-based programs may be valuable. Further research is needed to evaluate the feasibility and clinical benefits of such enhancements.

6 | Conclusion

This systematic review underscores the significant challenges faced by caregivers of children with ADHD, highlighting the pervasive effects of affiliate and courtesy stigma. These stigmas are linked to caregivers' mental health and social functioning but also predict their interactions and coping strategies. It is hypothesized that education and knowledge about ADHD could be crucial factors in reducing stigma, suggesting the need for targeted interventions to improve caregiver well-being. Despite the review's limitations, the literature remains clear: addressing caregiver stigmatization is essential for enhancing the overall well-being of caregivers of children with ADHD and their families.

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Ethics Statement

This systematic review did not involve any primary data collection or human participants. Therefore, ethical approval and informed consent were not required.

Conflicts of Interest

The authors declare no conflicts of interest.

Data Availability Statement

The data that support the findings of this study are available from the corresponding author upon reasonable request.

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